

Falls and Falling - Male (Child)

Algorithm 1351 | Child (0-17 years) | Gender at birth: Male

Definition

- Safety-first assessment after a fall in children and adults, including trauma, medical causes, anticoagulation, pregnancy, frailty, and safeguarding.
- Child patient.
- Male.

Initial assessment

1. [OPEN_TEXT] How, when, and from what height did the fall occur, and was there collapse, fainting, seizure, chest pain, palpitations, or weakness before it?
2. [YES_NO] Is the person awake, breathing normally, able to move all limbs, and safely able to get up without severe pain?
3. [YES_NO] Was there head, neck, back, hip, abdominal, or major limb injury, loss of consciousness, vomiting, confusion, memory loss, seizure, bleeding, or new weakness or numbness?
4. [OPEN_TEXT] Record age, pregnancy, anticoagulant or antiplatelet use, bleeding disorder, frailty, osteoporosis, intoxication, and whether the history or caregiver situation raises abuse, neglect, or safeguarding concern.
5. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Fainting

Did fainting or a temporary loss of consciousness occur before the fall?

A collapse caused by fainting belongs in the Fainting pathway.

1. Disposition level 100

Is the person unresponsive, not breathing normally, unable to get up, or is there suspected head, neck, back, hip, abdominal, or major limb injury; uncontrolled bleeding; severe pain or deformity; repeated vomiting, seizure, confusion, new weakness or numbness; or a fall associated with chest pain, collapse, or pregnancy trauma?

NHS.UK falls guidance lists possible head/back/neck/hip injury and inability to get up as emergency-assessment criteria.

1. Disposition level 70

Without an emergency feature, is there pain, swelling, a wound, dizziness, new unsteadiness, suspected medical cause, anticoagulant use, pregnancy, frailty, recurrent falls, or any child, older-adult, abuse, neglect, or unsafe-caregiver concern?

Injury may be occult and a fall may be caused by acute illness or medication. Higher-risk patients and safeguarding concerns need prompt assessment; exact Qatar routing is governance-required.

1. Disposition level 15

Was this a low-level accidental fall in a generally healthy adult who is now completely well, walking normally, with no pain, injury, head strike, medical trigger, high-risk medicine or condition, recurrence, or safeguarding concern?

NHS.UK guidance recommends routine primary-care review to discuss balance/mobility for repeated minor falls, and general fall-prevention self-care otherwise.

Advice

Emergency fall precautions

- Call Qatar 999 now. Do not move the person unless needed to escape immediate danger or directed by the call-handler; support the head and neck, keep them warm, control external bleeding without pressing on a deformity, give nothing by mouth, and do not allow self-driving. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: reduced consciousness, abnormal breathing, seizure, vomiting, weakness, or numbness; worsening pain, bleeding, or shock.

Prompt in-person fall review

- Use the Qatar governance-approved in-person service and timeframe to assess injury and the cause of the fall. Do not stop anticoagulants or other medicines unless instructed by the treating clinician. Address privacy and safeguarding without confronting a suspected unsafe caregiver.
- Call Back If: new headache, vomiting, confusion, weakness, numbness, chest pain, fainting, or inability to walk; pain, swelling, bleeding, or breathing difficulty worsens.

Fall prevention at home

- Stay physically active with regular strength and balance exercises, wear well-fitting shoes with good grip, keep a phone or alarm nearby, remove trip hazards and loose wires, use non-slip bath mats, and review medications with a primary-care clinician or pharmacist if balance is a concern. Arrange primary-care review if falls keep happening.
- Call Back If: falls become more frequent; pain or injury is noticed later.

First aid

- Call Qatar 999 now. Do not move the person unless needed to escape immediate danger or directed by the call-handler; support the head and neck, keep them warm, control external bleeding without pressing on a deformity, give nothing by mouth, and do not allow self-driving. Call Qatar 999 now, follow the call-taker's

ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Falls", <https://www.nhs.uk/conditions/falls/> (page last reviewed 06 March 2025)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: d8f98226aa38e637a8c73d8854fb90e93c10af15e0162897ca9529a501b855ca | Status: BLOCKED_BY_RESEARCH_GAP